

MOOT PROPOSITION

PART I

Swatantra Pradesh is a federal unit within the **Union of Andhrakisthan**. Though politically quasi-federal in matters assigned by the federal compact, its constitutional architecture, judicial structure, and administrative conventions operate in *pari materia* with the constitutional framework historically associated with the **Union of India**. The jurisprudence of fundamental rights, the doctrine of judicial review, and the interpretive traditions of public law remain deeply influential within its Courts. In the recent past the Constitutional Courts have been inspired by the interpretive approach of transformative constitutionalism, living constitution, dignity jurisprudence, and constitutional morality.

Geographically, **Swatantra Pradesh** stretches along the **Boromandal Coast**, where the shifting blues of the **Azure Sea** dissolve into the narrow historic passage known as the **Kalinga Strait**. Inland, the terrain rises gradually into the gentle shoulders of the **Easternly Ghats**, undulating hill ranges broken by fertile valleys, seasonal streams, and ancient forest settlements. Among these valleys lies **Ukara**, a region celebrated equally for its emerald coffee plantations and for the quiet endurance of indigenous tribal communities whose cultural memory predates recorded political history.

The socio-cultural fabric of the State is neither wholly ancient nor entirely modern. It is instead a layered continuum, locally described as **Telangam** culture, a synthesis of agrarian ritual, classical art, mercantile ambition, and technological aspiration. Fertile deltas nourished by the **Krishnaveni** and **Godavarneer** river systems sustain its dense rural populations, while port cities such as **Vishakshapuri** anchor shipbuilding, naval logistics, and maritime trade. At the same time, **Quantum Valley**, the State capital, has in the recent times emerged as a prominent corridor for biotechnology, artificial intelligence, and advanced medical research.

Classical expression remains deeply revered among all sections of the society. The intricate dance tradition of **Kuchipodi** and the devotional musical corpus known as **Annama Sangeetam** continue to be preserved through hereditary transmission, patronage institutions, and state-supported academies. Notably, certain musical compositions, particularly the rare “**Sarasa**” **kritis**, are believed within particular lineages to carry aesthetic nuances inseparable from familial inheritance. The High Court of **Swatantra Pradesh**, situated in **Quantum Valley**, exercises expansive constitutional writ jurisdiction under **Article 226** of the **Constitution of Andhrakisthan**, mirroring the remedial breadth historically associated with Indian constitutional courts. Questions touching dignity, identity, bodily autonomy, privacy, and technological ethics increasingly arrive before this Court, reflecting the tensions and synergies between inherited social norms and rapidly evolving biomedical science.

PART II

In the hill-edge settlement of Vaddesnagar in Rutnug District, reside Dr. Arvind Reddy, a cardiologist of considerable professional repute, and his wife Anjali Reddy, a classical Carnatic vocalist belonging to an old and respected musical lineage. Their marriage, solemnised fifteen years earlier, was widely regarded as a Union of Intellect and Art—medicine and music bound within shared cultural devotion. Despite sustained medical consultation for more than a decade after their marital knot, the couple remained unable to conceive naturally due to a documented reproductive condition affecting Anjali.

Within their familial and cultural milieu, the absence of a biological child carried emotional, social, and symbolic weight. Beyond personal longing, they wished for a successor capable of inheriting Dr. Reddy's medical legacy and Anjali's custodianship over a rare eighteenth-century archive of Sarasa compositions, transmitted exclusively through blood descendants for eleven generations.

After prolonged deliberation, the couple elected to pursue gestational surrogacy, a practice then governed primarily by medical guidelines and private contractual arrangements rather than comprehensive statutory regulation[1].

PART III

Lakshmi, aged twenty-eight in 2007, was employed as a nurse at the hospital where Dr. Reddy practised as a general physician. In the same year, she consented to function as a gestational surrogate for the Reddy family in return for monetary consideration of Rupai 8 lakhs (the currency of Andhrakisthan). Lakshmi belonged to a socio-economically vulnerable household in the old quarters of Vishakshapuri, and the consideration was expected to alleviate familial indebtedness and support the education of her siblings. The surrogacy arrangement, including the associated medical procedures and contractual formalities, was facilitated through Arogya Fertility Centre (AFC), which at the relevant time was regarded as a technologically advanced reproductive clinic in the State of Swatantra Pradesh.

Unknown to the intended parents and to Lakshmi, the sperm used in the procedure originated from an anonymous donor sample archived by AFC in 2006. The donor, whose identity was recorded only within restricted clinical files had executed a written confidentiality agreement stipulating permanent anonymity and the clinic had mutually agreed to protection of privacy and anonymity. The clinic director at the time, Dr. L. Priyalaxmi, authorised the use of the preserved sample in accordance with prevailing professional norms.

[1] At the relevant time, assisted reproductive arrangements in Andhrakisthan were not governed by any comprehensive statutory enactment and operated within a largely permissive legal vacuum, guided principally by private contractual arrangements and the Inland Council of Medical Research's National Guidelines for Accreditation, Supervision and Regulation of ART Clinics in Andhrakisthan, 2005, which were advisory in nature and lacked binding statutory force.

PART IV

On 12 January 2008, a female child was born through gestational surrogacy and was named Aariv Reddy. The birth certificate recorded Dr. Arvind Reddy and Anjali Reddy as her legal parents. Lakshmi relinquished all claims in accordance with the agreement and returned to her native village to start a new life. Within the clinic's internal records, however, a brief handwritten annotation described the donor:

"Age: 32; Blood group: B+; Telangam Brahmin lineage; Engagement in "party work"; No family history of cardiac disease"

PART V

Aariv was brought up in an environment saturated with music, scholarship, and disciplined affection. She aspired to pursue professional legal education and learn her family's traditions with greater enthusiasm. In particular, she intended to seek admission to the Bar through Lincoln's Inn (United Kingdom). By the age of fourteen, she began formal training in the Sarasa tradition under her mother's guidance. Yet subtle dissonances emerged for certain melodic transitions, traditionally believed to arise intuitively within the lineage, proved persistently difficult for her. Her music instructor remarked privately, though without malice, that artistic inheritance sometimes followed "paths invisible to pedagogy."

During a school genetics project, Aariv learned that her blood group was B+, while both her recorded parents possessed O+ blood groups. The inconsistency prompted questioning to which her parents, for the first time, disclosed the fact of surrogacy, but assured her she remained genetically theirs. The matter rested uneasily, neither fully explored nor entirely forgotten.

PART VI

In January 2024, Dr. Arvind Reddy suffered sudden cardiac arrest while he was working. He was diagnosed with a rare hereditary disorder identified in regional medical literature as Telangam Cardiomyopathy Variant (TCV), a condition associated with particular sub-communities. He was given the best treatment available and required constant personal monitoring and intermittent medical attention thenceforth. For Aariv, the diagnosis transformed curiosity into fear. Her own medical risk could not be evaluated without clarity regarding paternal genetic history.

In March 2025, while reviewing archived medical files of the family, Aariv encountered references to "Donor No. 32." Confronted again, her mother disclosed the complete truth. The disclosure revealed that the ovum was Anjali's while the sperm had originated from an anonymous donor.

PART VII

Upon attaining majority, on 12 January 2026, Aariv formally approached Arogya Fertility Centre, seeking the disclosure of the donor's identity, or, at minimum, his genetic and medical history. The clinic refused the request, citing:

1. The 2007 confidentiality agreement (claiming that the disclosure would tantamount to breach of contract)
2. Emerging statutory policy reflected in the Surrogacy (Regulation) Act, 2021, and privacy concerns.
3. Potential harm to the donor, now described as a public figure.

Fragments later surfaced through informal inquiry: the donor had been a young Telangam political worker undergoing treatment at the hospital during 2006-07.

PART VIII

The donor was eventually inferred to be Srikanth Bhagavatar (aka SK), now aged fifty-two and a Cabinet Minister for the Department of Happiness in the State. He is an anticipated chief-ministerial candidate, public advocate of traditional family values, and a vocal critic of anonymous reproductive technologies. He is married with two children, none of whom is aware of any prior sperm donation. Upon receiving a copy of legal notice, he instructed his counsel to oppose disclosure on grounds of absolute contractual confidentiality, right to privacy and family life, and irreparable personal and political harm.

PART IX

On 12 March 2026, Aariv experienced cardiac symptoms, physical discomfort, shortness of breath, and palpitations. Her cardiologist recorded:

"Definitive diagnosis of suspected TCV requires paternal genetic history. Absence of such information prevents risk assessment, preventive care, and counselling regarding future offspring." She apprehended that the complicated state of health and lack of information could entail risks to physical health and affect the prospects of her reproductive rights. Medical urgency thus intersected with constitutional silence. She began mulling options of legal recourse and asserted that within Telangam tradition, Gothram determined ritual belonging, lineage shapes marital legitimacy, and artistic inheritance carries genealogical meaning. If her paternal origin differed from assumed lineage, she argued, her cultural self-understanding remained incomplete.

PART X

A writ petition under Article 226, was filed by Aariv on February 14, 2026, which inter alia sought:

- Recognition of a fundamental right to know one's genetic origins under Article 21 and protection of her right to health.
- Disclosure of the donor's identity.
- Alternatively, disclosure of medical/genetic data via neutral mechanism.
- Interim genetic testing without public revelation

The petition arrayed the following as respondents: Arogya Fertility Centre, Srikanth Bhagavatar (as a potential appropriate/necessary party, though she had no definitive answer to her search yet), State of Swatantra Pradesh, Union of Andhrakisthan.

In their preliminary objections, the Respondents (especially 1 and 2) contended that any compelled disclosure of genetic or biological identity would constitute a serious infringement of the right to privacy and decisional autonomy as recognised by the constitutional courts. It was further argued that the surrogacy arrangement and all attendant medical records were governed by binding contractual and fiduciary obligations of confidentiality, the breach of which would expose the parties and the medical institution to civil and regulatory liability. The Respondents also asserted that disclosure at a belated stage would cause irreparable harm to personal dignity, familial stability, and social reputation, particularly in the sensitive context of assisted reproduction. Emphasis was placed on the child's best interests, including the preservation of psychological security and protection from unwanted public scrutiny. Additionally, it was submitted that retrospective interference with a concluded reproductive arrangement would undermine legal certainty in emerging fields of reproductive technology and deter legitimate medical practice. On these grounds, the Respondents maintained that the reliefs sought were constitutionally impermissible, contractually barred, and contrary to settled principles governing privacy, reputation, and the welfare of the child. However, the petitioner argued against the retrospectivity of the Constitutional rights, especially recognised through judicial precedents, inadequacy of other existing evidentiary principles within the statutory scheme, and need for a judicial intervention.

PART XI

As the matter proceeded to final hearing, the Court framed the following issues for consideration at the stage of oral arguments scheduled for 2 and 3 April 2026:

1. Whether the right to know one's genetic origins forms part of the right to life and personal liberty under Article 21.
2. Whether a donor's right to privacy may override a donor-conceived adult's medically necessary claim to genetic information.
3. Whether contractual confidentiality in assisted reproduction can supersede constitutional protections of dignity, identity, and health.

Note: The participating teams shall refrain from framing additional issues. However, allied issues, questions and concepts of law governing the issues above may be argued as sub-issues.